

DISCHARGE SUMMARY — CARDIAC CARE

Patient	Arjun Mehta	Age/Sex	42 Years / Male
IP No.	KAU-2024-06213	Ward	Cardiac ICU → Step-down
Admitted	18-Sep-2024	Discharged	28-Sep-2024
Consultant	Dr. Venkat Rao, DM DM Cardiology	Dept.	Interventional Cardiology

DIAGNOSIS

- Acute ST-Elevation Myocardial Infarction (STEMI) — Anterior wall
- Primary PCI performed — LAD stent (Drug-Eluting Stent, 3.0x28mm)
- Systolic Dysfunction — EF 35% post-MI (reduced from presumed normal)
- Dyslipidaemia — LDL 178 mg/dL at admission
- Hypertension — newly diagnosed
- Smoker — 20 pack-years. Counselling for cessation.

HISTORY

42-year-old male, software company director. Active smoker. No prior cardiac history. Presented with acute crushing central chest pain radiating to left arm, diaphoresis, and vomiting for 90 minutes. ECG: ST elevation V1-V6. Troponin I: 48.2 ng/mL (massively elevated). Taken for emergency primary PCI within 45 minutes of arrival.

ALLERGY HISTORY

No known drug allergies. No prior surgeries or hospitalisations.

PROCEDURE DETAILS

Primary PCI on 18-Sep-2024:

- Culprit vessel: Left Anterior Descending (LAD) — 100% occlusion at mid segment
- Procedure: Thrombus aspiration + balloon angioplasty + DES implantation (3.0x28mm)
- Post-procedure TIMI flow: Grade 3 (normal)
- Right coronary artery: 40% stenosis — medical management, no intervention

INVESTIGATIONS

Test	Admission	Day 3	Day 7	Status
Troponin I (ng/mL)	48.2	21.4	2.1	Falling — expected
CK-MB (U/L)	284	142	18	Falling
Echo EF	35%	38%	40%	IMPROVING — still reduced
LDL Cholesterol	178 mg/dL	—	—	HIGH ↑↑
Blood Sugar (FBS)	142 mg/dL	—	98 mg/dL	Stress hyperglycaemia — normalised
HbA1c	5.8%	—	—	NORMAL (not diabetic)
Creatinine	1.0 mg/dL	—	1.1 mg/dL	NORMAL

DISCHARGE MEDICATIONS — DUAL ANTIPLATELET + CARDIAC

Medication	Dose	Duration	Critical Notes
Aspirin	75mg OD	LIFELONG	Do NOT stop without cardiologist advice
Clopidogrel	75mg OD	12 months minimum	DAPT — stent thrombosis risk if stopped early
Atorvastatin	80mg OD	LIFELONG	LDL target < 55 mg/dL for STEMI
Bisoprolol	5mg OD	Long term	Post-MI EF < 40% — cardioprotective
Ramipril	5mg OD	Long term	ACE inhibitor — EF recovery
Eplerenone	25mg OD	3 months	Aldosterone antagonist — EF < 40%
Nitroglycerin spray	400mcg	SOS	For chest pain — carry at all times
Nicotine patches	21mg/24hr	8 weeks	Smoking cessation

CRITICAL FOLLOW-UP INSTRUCTIONS

DO NOT STOP Aspirin or Clopidogrel for ANY reason (including minor surgery) without cardiology clearance. Stent thrombosis can be fatal.

- Cardiology OPD in 4 weeks — repeat Echo, stress test
- Cardiac Rehabilitation Program — 12-week structured exercise programme
- Complete smoking cessation — MANDATORY
- Repeat Echo in 3 months — if EF > 45%, consider stopping Bisoprolol
- If any chest pain / breathlessness → EMERGENCY — call 108 immediately